Kaya

A multi-organ digital twin for Type 2 Diabetes

It shows what glucose does to the liver, eyes and kidneys, and lets a doctor test a change first.

Team Kaya · IIT Hyderabad · Dulari Bhushan, Prateek Giddaplavar

Happiest Health Reimagining and Reforming Healthcare in India Summit 2026 · Digital Twin Challenge, Phase 1

The damage from diabetes is silent

Glucose is tracked. The organs it harms are checked once a year, if at all.

100M+

people with diabetes in India (ICMR-INDIAB)

21.7%

of Indians with diabetes had retinopathy (AIOS screening)

22%

of US adults with diabetes have advanced liver stiffness (NHANES)

- Fatty liver, retinopathy and kidney disease build for years before symptoms.
- Existing twins and apps forecast sugar and coach diet. None we reviewed links it to organ damage.

What exists, and the gap

Reviewed from public sources

Who	What it does	Gap
Twin Health	Remission program: CGM, wearables, care team	Reports liver and weight benefit; closed program
Fitterfly, sugar.fit, Ultrahuman	CGM plus coaching and scores for Indian users	Glucose and lifestyle; no organ outcomes
Dexcom, Roche SmartGuide	Predictive low-glucose alerts 20-30 min ahead	Glucose only
Remidio + RetinaRisk	Offline smartphone DR screening, risk-based interval	Eye only; no wearable link
ARTEMIs, HEPATWIN (EU)	Virtual organ twins for liver disease	Research stage; liver only
Kaya	One twin: glucose to liver and eye, plus what-if	**Open; validated per module**

What Kaya does

Two data streams fused into one twin, four questions answered, one place to test a change

Static: labs, genetics, history + Dynamic: CGM, heart rate, sleep, steps, meals

Glucose

What will glucose do in the next 2 hours? Is an event coming?

Liver

Is fat building up? Is fibrosis developing?

Eye

What is the retinopathy grade and where is it heading?

Kidney

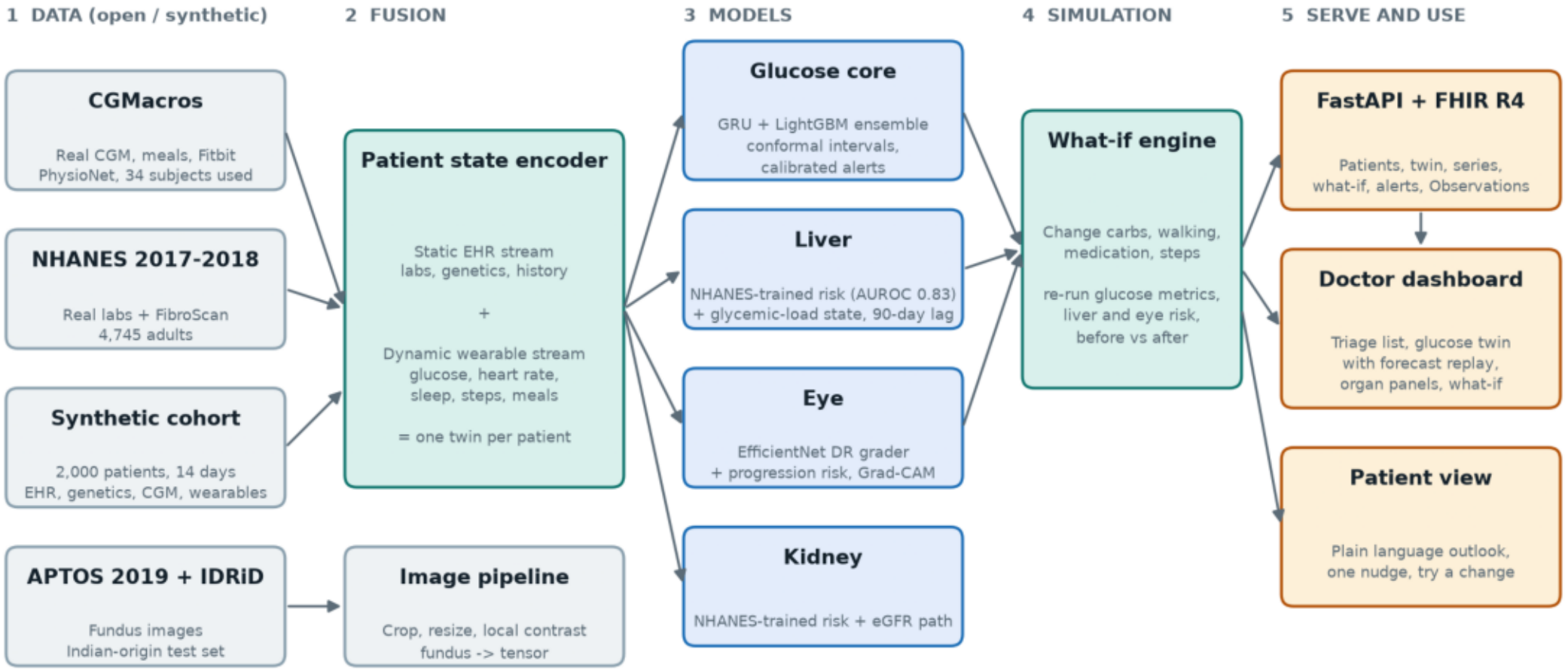
Is kidney function declining, and how fast?

What-if engine

Change carbohydrates, walking, medication or steps, and see the glucose curve and one-year organ risk move

Kaya: system architecture

Multi-organ digital twin for Type 2 Diabetes. Open and synthetic data only.



Validation: patient-level splits, real-data checks per module, calibration, ablations | Governance: synthetic or open data only (DPDP-aware), decision support not diagnosis, model cards

Data: open or synthetic only

What is real, what is simulated, and how we validate

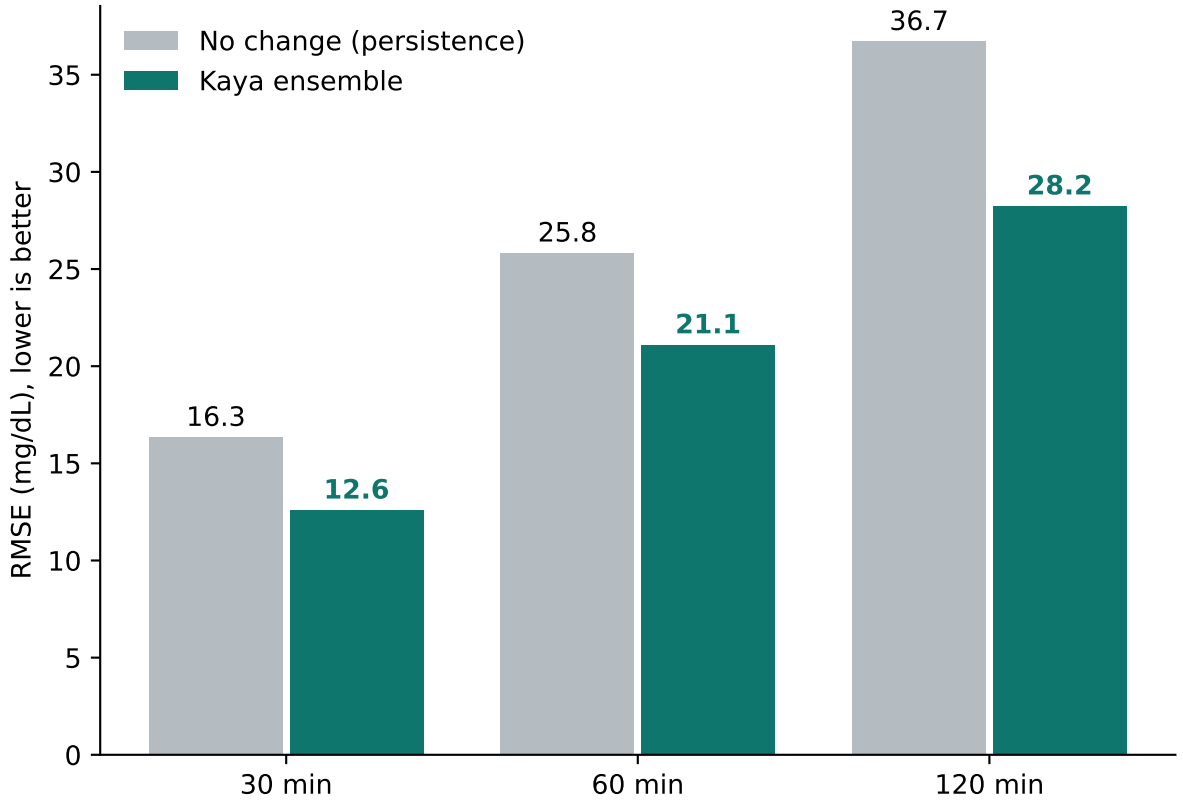
CGMacros (PhysioNet)	**Real**	Glucose forecaster: 34 subjects, subject-level cross-validation
NHANES 2017-2018	**Real**	Liver model: 4,745 adults with FibroScan labels
APTOS 2019 + IDRiD	**Real**	Retina grader: train on APTOS, test on Indian IDRiD
Synthetic cohort	**Simulated**	2,000 patients: links glucose patterns to organs for the twin and what-if

Why this is honest

No public dataset links one person's glucose to liver, eye and kidney outcomes. Each module is checked on real data where it exists.
The glucose-to-organ links and what-if effect sizes are editable, literature-informed assumptions, labelled as simulation in the product.

Glucose forecasting on real, held-out subjects

CGMacros, 34 subjects, subject-level 5-fold cross-validation



72%

of low-glucose episodes caught

45 min

median warning before the low

80%

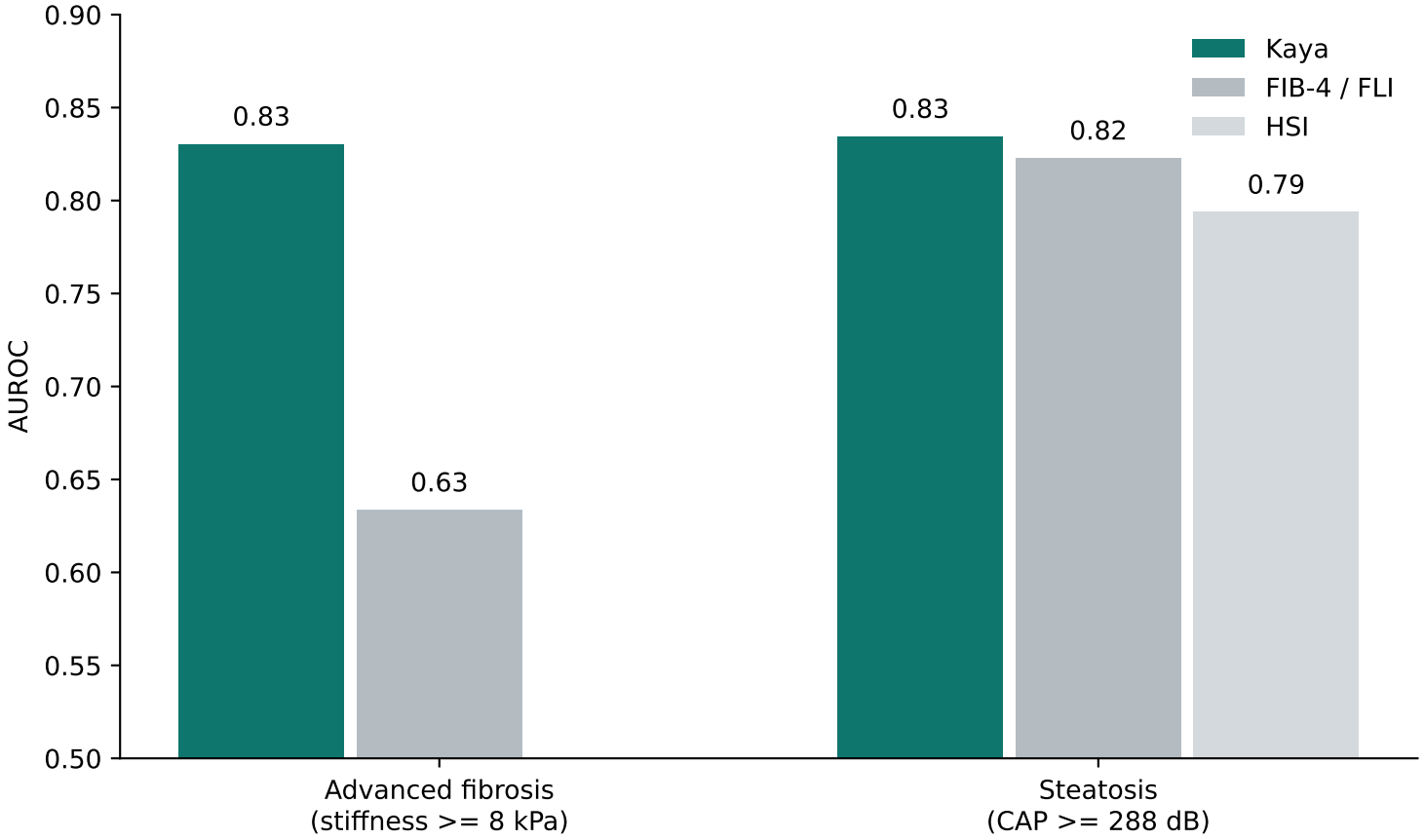
interval coverage (conformal, exact)

~1/day

false alerts per patient at that operating point

Liver: validated on real FibroScan data

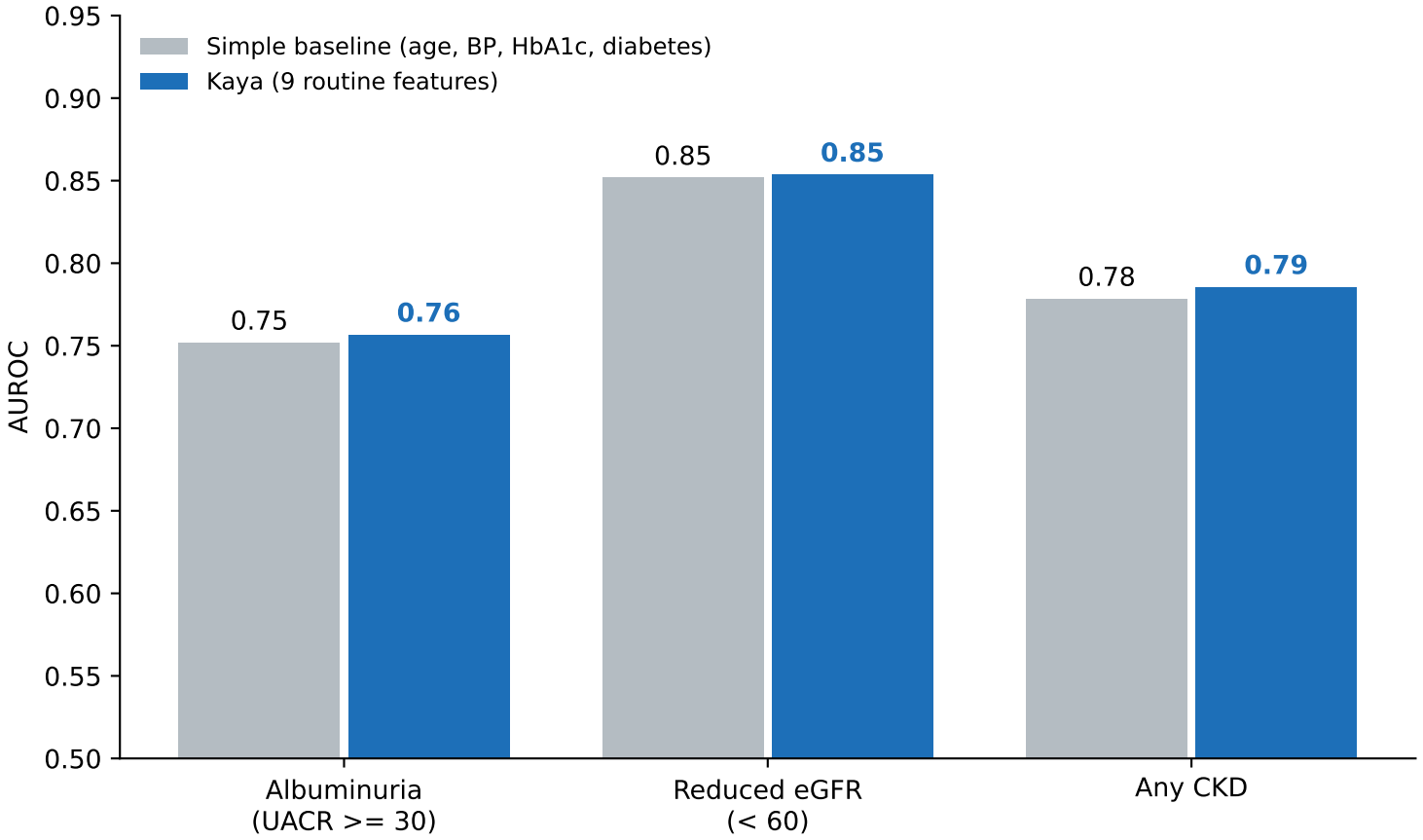
NHANES 2017-2018, 4,745 adults, 5-fold cross-validation, 95% CIs in the repository



- Fibrosis: 0.83 vs 0.63 for FIB-4
- Steatosis: 0.83 vs FLI 0.82, HSI 0.79
- Holds in adults with diabetes (0.76 / 0.80) and in A
- On top: a transparent glycemic-load state that mo

Kidney: real NHANES data, with honest limits

4,827 adults, 5-fold cross-validation; risk from routine measurements (no creatinine used)



- Reduced eGFR 0.85, albuminuria 0.76
- Adults with diabetes: 0.77 and 0.70
- Honest: only a small edge over a simple baseline, which is already strong
- On top: an eGFR path (about 1 to 4 mL/min per year) driven by HbA1c, BP, albuminuria and variability, as an editable simulation

Eye: retinopathy grading, tested on Indian images

EfficientNet-B2 trained on APTOS 2019, tested on IDRiD (Indian), ordinal grading

0.89

quadratic kappa
APTOS held-out

0.98

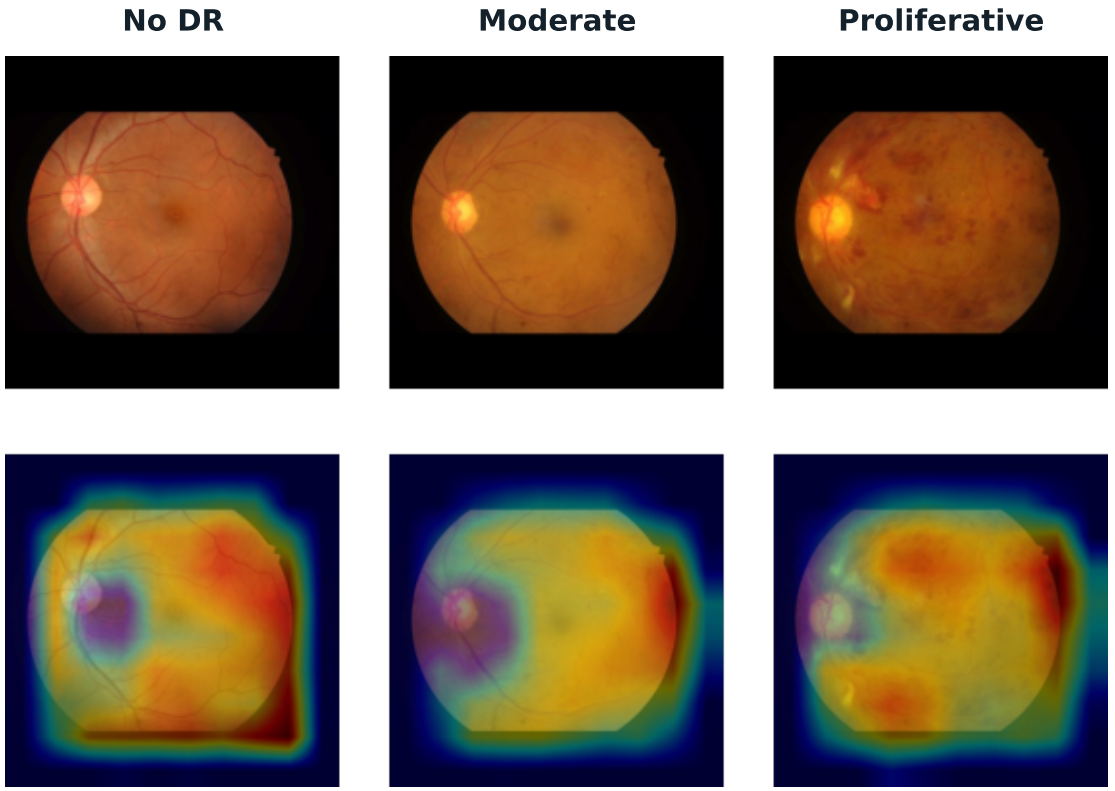
referable DR AUROC
APTOS held-out

0.71

quadratic kappa, IDRiD test
(Indian, fine-tuned locally)

0.92

referable DR AUROC
IDRiD test



Top: IDRiD fundus. Bottom: Grad-CAM, where the model looked.

Without fine-tuning on IDRiD: kappa 0.57, AUROC 0.88. Camera and population shift is real, and local data closes part of the gap.

What-if: test the change on the virtual patient

Patient 1705, last 14 days replayed with three changes together

	Today	With the change
Time in range	42%	**71%**
Time above 180	58%	**29%**
Liver steatosis risk, 12 months	26%	**11%**
Advanced fibrosis, 12 months	35.5%	**24.4%**
Retinopathy progression, 2 years	6%	**2%**

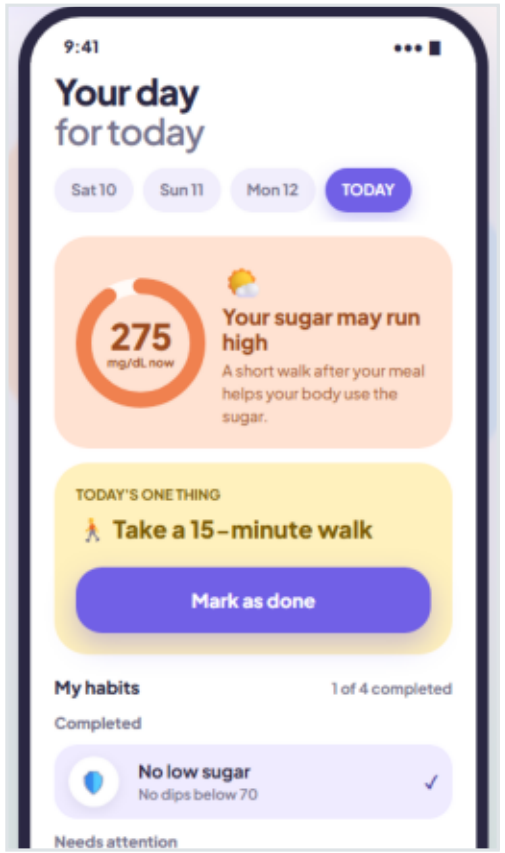
Walk 20 min after meals, 25% smaller carbohydrate portions, medication optimised. Effect sizes are editable, literature-informed assumptions and shown to the clinician as a simulation, not a prediction.

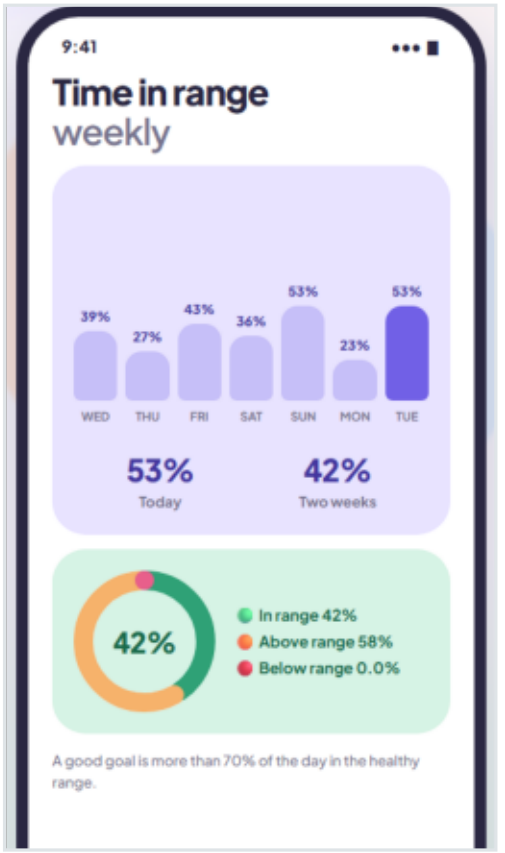
The product

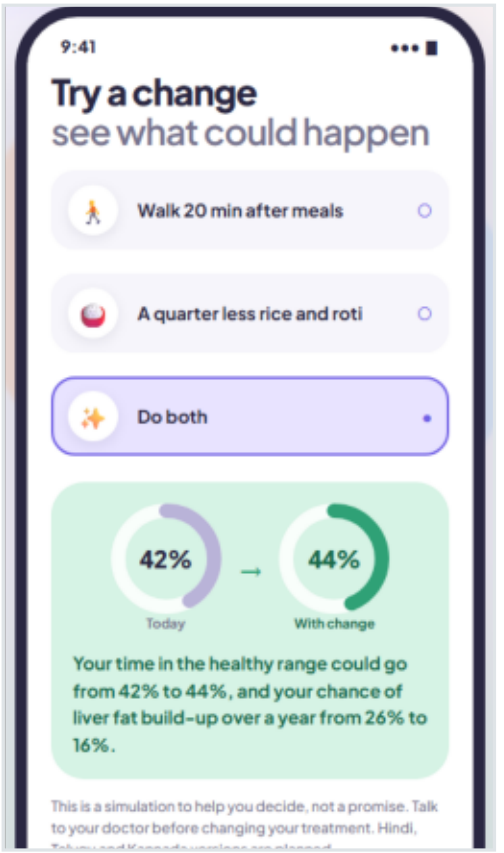
Doctor workspace and patient view. Replay shows the forecast against what actually happened.



Doctor workspace







Patient app: today, insights, try a change

Limitations, ethics and what is next

Limitations we state openly

- Glucose-to-organ links and what-if effect sizes are assumptions, not clinical findings
- Glucose model validated on 34 real subjects; patients in the demo are synthetic
- Decision support for clinicians, not a diagnostic device; not clinically validated

Roadmap

- An illustrative, clearly labelled retina progression preview and a blood-pressure what-if
- Hindi, Telugu and Kannada patient view; PDF clinical summary
- Consent records, audit log, role-based access, fuller FHIR and ABDM alignment
- Validate the glucose-to-organ links on real longitudinal data when available

Existing twins manage sugar.

Kaya shows what sugar does to the organs, and lets a doctor test the fix first.

Open source, Apache-2.0 · open and synthetic data only · github: see README

Team Kaya · IIT Hyderabad · Dulari Bhushan, Prateek Giddaplavar